INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Bevacizumab 10 · colorectal

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
14 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00212
Version 7

> Bevacizumab Day 1 every 14 days, with separately prescribed chemotherapy. Continue while helping and tolerable; no fixed final cycle.

Treatment stages

01 **Bevacizumab with chemotherapy**
14-day reference cycle

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call your oncology team immediately for coughing blood, significant bleeding, severe tummy pain, sudden breathlessness or chest pain, or a new severe headache, confusion, seizure or vision change. Fever or sudden illness also needs urgent advice, especially with chemotherapy.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Three medicines. Different jobs.

To help control bowel cancer that has spread, alongside the chemotherapy chosen by your oncology team.

ANTI-VEGF ANTIBODY

Bevacizumab

Bevacizumab



Blocks the VEGF growth signal

Bevacizumab binds VEGF, a signal that helps new blood vessels grow. It can slow tumour growth but also affect normal blood vessels and wound healing.

Blocks the VEGF growth signal → Cancer growth disrupted → Healthy tissues may also be affected

What to watch for

Report bleeding, severe tummy pain, unusual headache or vision changes urgently. Tell the team about any surgery or wound.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why bevacizumab is included

This is one part of your combination treatment.

Which evidence applies?

E3200 studied bevacizumab 10 mg/kg every two weeks with FOLFOX4 in metastatic colorectal cancer previously treated with a fluoropyrimidine and irinotecan. Adding bevacizumab improved survival in that trial. This does not establish the same result with every chemotherapy partner or predict an individual outcome.

Evidence in this colorectal setting

The chemotherapy partner, dose schedule and previous treatment determine how trial results apply.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Treatment → review → next steps

Bevacizumab 10 · colorectal

14-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

14-DAY REFERENCE CYCLE

Bevacizumab with chemotherapy

Review

Bevacizumab is given by drip on Day 1 every 14 days with the prescribed fluoropyrimidine-based chemotherapy. Treatment continues while helping and tolerable; there is no fixed total number of cycles. The partner chemotherapy has its own dose days, tests and support plan.

Your team confirms each stage and discusses any changes with you.

BASELINE → TREATMENT → FOLLOW-UP

My treatment and safety checks

Routine safety tests are separate from scans and tumour markers, which follow your individual plan. No universal imaging or tumour-marker interval is specified.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00212 v7 · official regimen · Avastin current EMA product information

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Bevacizumab 10 · colorectal: treatment and checks

Metastatic colorectal cancer · bevacizumab component with fluoropyrimidine-based chemotherapy

Bevacizumab is given by drip on Day 1 every 14 days with the prescribed fluoropyrimidine-based chemotherapy. Treatment continues while helping and tolerable; there is no fixed total number of cycles. The partner chemotherapy has its own dose days, tests and support plan.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before treatment	Treatment / review	Between visits	Phase decision	After treatment
Treatment NCCP 00212	Confirm colorectal indication / partner	Bevacizumab Day 1 every 14 days	Follow partner prescription	Review benefit and tolerability	Next plan agreed individually
Bloods / urine NCCP 00212	full blood count (FBC), kidney/liver, urine protein	Regular blood/urine checks	Quantify protein if needed	Review trends	Follow abnormalities
Blood pressure / heart NCCP 00212	blood pressure (BP) and cardiac history/exam	BP before each cycle	Extra BP if raised	Review control	BP post-treatment
Scans / other tests NCCP 00212	Individual disease plan	No fixed scan interval	international normalised ratio (blood-clotting test) (INR) if warfarin	Response review	Follow-up agreed

What each check is for

1 Bevacizumab with chemotherapy

14-day reference cycle

To help control bowel cancer that has spread, alongside the chemotherapy chosen by your oncology team.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Partner chemotherapy

This covers bevacizumab only. Bring the separate chemotherapy schedule, anti-sickness plan and urgent-contact instructions.



STEP 2

Blood pressure and urine

These checks can find problems before you feel unwell. Take blood-pressure medicines as prescribed; do not adjust them yourself.

STEP 3

Surgery and dental procedures

Tell oncology before any operation or invasive procedure. The team must agree treatment pauses and assess wound healing.



STEP 4

Support medicines

Bevacizumab alone does not usually need routine anti-sickness prevention. Chemotherapy partners may need a different plan.

Before treatment starts

Some problems are found by tests before symptoms develop.

Baseline checks

Blood count, kidney and liver tests, urine protein, blood pressure and a heart history/examination are checked. A heart ultrasound is considered after chest-wall radiotherapy or previous anthracycline treatment.

Medicines, wounds and procedures

Bring all medicines and supplements. Report bleeding, clotting problems, poorly controlled blood pressure, unhealed wounds and planned operations or dental procedures. Oncology must coordinate any treatment pause.

The complete combination

Confirm the chemotherapy partner, its tests, infusion or tablet calendar, any pump arrangements and prescribed anti-sickness plan. This bevacizumab pack cannot replace those instructions.

Review and follow-up

There is no universal final cycle or scan timetable.

Assessing benefit

Your consultant sets scans and any tumour-marker tests according to your cancer, symptoms and treatment plan. These are separate from routine blood-pressure and laboratory safety checks.

After a pause or stopping

Continue arranged blood-pressure checks and follow-up of unresolved effects. Ask who reviews results, what treatment follows and whom to contact if symptoms develop.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 14: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Bevacizumab infusion	 **Day 1** Day 1 every 14 days after review
Separate chemotherapy schedule	 **Days 1–14** Use the named partner pack for tablet days, infusions or pump timing
Symptoms and safety checks	 **Days 1–14** Symptoms can occur at any time; attend arranged tests

Partner treatment changes the schedule: This calendar shows bevacizumab only. Fluoropyrimidine-based chemotherapy may involve tablets, other infusion days or a home pump. Use its separate written calendar.

Treatment decisions: The team may hold or stop bevacizumab for side effects. Decisions for each chemotherapy medicine are separate; do not restart or alter treatment yourself.

Symptoms can occur at any time: Raised blood pressure or protein in urine may have no symptoms. Attend tests, and report bleeding, severe abdominal pain, chest pain, breathlessness or new neurological symptoms promptly.

> **Why these bands stay broad:** Relative days without personal dates. The team confirms actual treatment and test dates. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment.

> **There are no guaranteed "safe days".** Call your oncology team immediately for coughing blood, significant bleeding, severe tummy pain, sudden breathlessness or chest pain, or a new severe headache, confusion, seizure or vision change. Fever or sudden illness also needs urgent advice, especially with chemotherapy.

Between-treatment notes

Day 1: Infusion and checks

Bevacizumab after review; the chemotherapy partner follows its own prescription. Confirm blood pressure, urine testing and the full combination plan.

Day 2–14: Between bevacizumab visits

Follow the separate chemotherapy calendar and report new symptoms promptly. Keep arranged checks; contact oncology if unwell.

Support around the medicines

1 **Blood pressure and urine**

2 **Support medicines**

During and between cycles

Your next dose depends on review, not just the date.

Blood pressure and urine

Blood pressure is checked before each cycle and after treatment. Regular blood-count, kidney/liver and urine-protein tests continue. Extra tests are arranged for abnormal results.

Warfarin

If taking warfarin, INR is checked weekly until the warfarin dose is stable, then before each cycle. Tell the team about bleeding or any medicine changes.

Support medicines

Bevacizumab alone has minimal sickness risk. The chemotherapy partner determines additional anti-sickness treatment and other supportive medicines; use the issued prescription.

Living with treatment

Keep information and contacts available on paper as well as online.

Before any procedure

Tell every treating clinician that you receive bevacizumab. It can affect wound healing; your oncology team must agree timing around operations and check healing before restarting.

Keeping a record

This is a rolling record, not a six-cycle limit. Record actual doses and interruptions. A stamp records completed treatment and does not authorise the next dose.

Pregnancy and fertility

Discuss fertility and pregnancy prevention with your team before treatment. Check the duration of contraception and breastfeeding restrictions for every medicine in the combination; contact oncology promptly if pregnancy is possible.

Questions worth asking

Who confirms my next dose?

Your treating team checks the prescription, tests and symptoms. A completed-cycle stamp records past treatment; it does not authorise the next dose.

What if the plan changes?

Ask the team to update your phase, medicines, test appointments and cycle record. Use the updated prescription.

When are my scans?

Your team sets a response and follow-up plan for your cancer and symptoms. This document does not invent a fixed scan or tumour-marker timetable.

> **Try explaining the plan back:** Which medicines are you having now? What changes after induction? What would make you call today? Ask your team to fill any gaps.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Bevacizumab component: Day 1 every 14 days

Active medicines: Bevacizumab

Emetogenic risk: Minimal (bevacizumab component only)

No routine antiemetic prophylaxis for the bevacizumab component. The prescribed fluoropyrimidine-based chemotherapy requires its own antiemetic and supportive-care plan; minimal risk here does not classify the whole combination.

Additional phase-specific support

- Premedication is not usually required unless there has been previous hypersensitivity; the treating team specifies any individual plan.
- Review blood pressure, urine protein, bleeding, thrombosis, wounds and planned procedures before dosing.
- Provide a prescribed diarrhoea plan appropriate to the chemotherapy partner; bevacizumab does not define a universal loperamide schedule.
- Urgent assessment for significant bleeding, severe abdominal pain, chest pain, sudden breathlessness, severe headache, confusion, seizure or visual disturbance.
- Use the separate chemotherapy schedule and support prescription. No lung six-cycle limit or ovarian course cap applies to this colorectal context.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Head / blood pressure

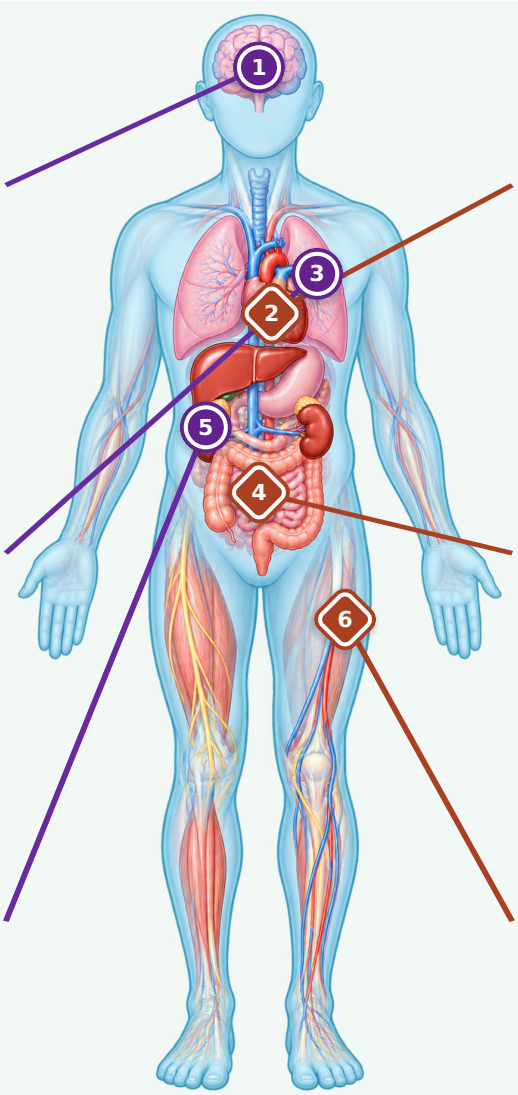
Bevacizumab

New severe headache, confusion or visual change needs urgent advice.

2 Heart / blood vessels

Bevacizumab

Chest pain, sudden breathlessness or a swollen painful leg needs urgent help.



3 Lung bleeding

Bevacizumab

Coughing up blood needs immediate assessment.

4 Bowel / abdomen

Bevacizumab

Severe tummy pain or a rigid swollen abdomen needs urgent assessment.

5 Urine / kidneys

Bevacizumab

Urine tests detect protein; report swelling or much less urine.

6 Bleeding / wounds

Bevacizumab

Report bleeding and unhealed wounds; tell the team before procedures.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call your oncology team immediately for coughing blood, significant bleeding, severe tummy pain, sudden breathlessness or chest pain, or a new severe headache, confusion, seizure or vision change. Fever or sudden illness also needs urgent advice, especially with chemotherapy.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00212 v7 · official regimen**
   https://healthservice.hse.ie/documents/6552/212_Bevacizumab_10.pdf
2. **Avastin current EMA product information**
   https://www.ema.europa.eu/en/documents/product-information/avastin-epar-product-information_en.pdf
3. **NCCP antiemetic classification v 6 and oral pathway**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf
4. **NCIS medical oncology antiemetic medicines**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
5. **E3200: bevacizumab with FOLFOX4 after previous treatment**
   https://pubmed.ncbi.nlm.nih.gov/17442997/
6. **NCI: colon cancer treatment — carcinoembryonic antigen (CEA) and follow-up**
   https://www.cancer.gov/types/colorectal/hp/colon-treatment-pdq